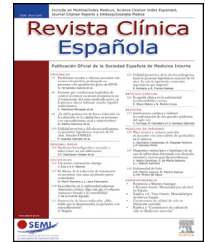




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CONSENSUS DOCUMENT

Executive summary of the 2025 consensus on the diagnosis and treatment of hypertension of the International Forum on Internal Medicine



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KEYWORDS

High blood pressure;
International
consensus;
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Treatment

Abstract The diagnosis and treatment of hypertension is a challenge for any healthcare system. Given the difficulties, limitations, and differences among Spanish- and Portuguese-speaking countries, the International Forum of Internal Medicine (FIMI) has promoted this Consensus, which includes 23 scientific societies from 21 countries in Europe and the Americas. The objective was to develop a proposal capable of establishing an updated, robust, and common framework for the diagnosis and treatment of hypertension, aimed at internists and hospitalists and useful in Latin America, Spain, and Portugal. The document we present includes the executive summary of FIMI recommendations that, for the various aspects of the disease, aim to guarantee effective, safe, efficient, sustainable, and proportionate healthcare interventions based on the best available scientific evidence. The authors consider that this document should be updated within a maximum of two years.

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¹ The numbers of components of the Arterial Hypertension Working Group-FIMI are related to Annex 1.

PALABRAS CLAVE

Hipertensión arterial;
 Consenso
 internacional;
 Prevención;
 Tratamiento

Resumen ejecutivo de consenso 2025 en diagnóstico y tratamiento de la hipertensión arterial del Foro Internacional de Medicina Interna

Resumen El diagnóstico y tratamiento de la hipertensión arterial (HTA) es un reto para cualquier sistema sanitario. Dadas las dificultades, limitaciones y diferencias entre los distintos países de habla hispano y lusa, el Foro Internacional de Medicina Interna (FIMI) ha promovido este consenso que incluye a 23 sociedades científicas de 21 países de Europa y América. El objetivo fue realizar una propuesta capaz de establecer un marco actualizado, sólido y común sobre el diagnóstico y tratamiento de la HTA dirigido a los médicos internistas y hospitalistas y útil en Latinoamérica, España y Portugal. El documento que presentamos recoge el resumen ejecutivo de recomendaciones del FIMI que, para los distintos aspectos de la enfermedad, pretenden garantizar intervenciones sanitarias efectivas, seguras, eficientes, sostenibles y proporcionadas y basadas en la mejor evidencia científica disponible. Los autores consideran que este documento debe actualizarse en el plazo máximo de 2 años.

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Introduction

Hypertension (HT) is the most frequent modifiable cardiovascular risk factor and is the main cause of premature mortality and disability worldwide.^{1,2} Internists, with their global vision, play a fundamental role in both the diagnosis and treatment of HT, providing comprehensive management in different clinical scenarios such as in consultations, during hospitalization, and in the emergency department.³

Addressing its complexity is key to providing answers to a highly important problem. However, in routine clinical practice, there is heterogeneity in care among different countries for many reasons. These include certain disparities in the most common clinical practice guidelines, in the cut-off values that define the disease, for example, or the resources available in the different health systems, especially in Latin American countries. The International Forum of Internal Medicine (FIMI), which includes all countries of Latin America, Spain, and Portugal, considers it essential to provide updated information on the diagnosis and treatment of HT based on the best available evidence that is focused on the situation in countries whose internal medicine societies are members of FIMI.

The need to establish a robust, common framework for internists on the diagnosis and treatment of HT that is useful in Latin America, Spain, and Portugal.

Methods

In October 2024, the FIMI board formed a working group of 54 physicians from 23 scientific societies in 21 countries (Annex 2). The general aim was to propose a general framework that allows for the diagnosis and treatment of HT that would be useful to FIMI member countries. The secondary aims included:

a Agree on a proposal of diagnostic methods.

b Update assessments of damage secondary to this condition.

c Design possible feasible treatment regimens that overcome existing barriers.

This work began as a narrative review and a FIMI Proposal of Recommendations was produced. The search, which was up to November 2024, was conducted in the TRIP Database, PubMed, Embase, and LILACS. Furthermore, the references included in the documents obtained were traced and experts were contacted. The document was produced through videoconference meetings held weekly from June to November 2024. The DELPHI methodology was used for the consensus. Three rounds were conducted, with the participation of the 54 experts who responded to a questionnaire (Likert scale with 5 options). Consensus was defined as when the percentage of "agree" or "strongly agree" responses were $\geq 75\%$ and $\geq 90\%$, respectively. The final document is available on the FIMI website: <https://webfimi.meducar.com/noticia/consenso-2025-en-diagnostico-y-tratamiento-de-la-hipertension-arterial-del-foro-internacional-de-medicina-interna/52267>

Results

The main recommendations are summarized below in Tables 1–6. The original document is extensive and there are aspects of the recommendations that could not be included in this executive summary. These are available in the full document. Fig. 1 shows the general strategy for pharmacological treatment with combination therapy.

Discussion

This document reflects FIMI's position in 2025 on the diagnosis and treatment of hypertension and updates versions previously published by this organization since 2018.³ This edition includes a new section on the management of

Table 1 Initial diagnosis and classification.

DIAGNOSIS Establish the diagnosis of hypertension by considering the mean blood pressure (BP) values obtained in two or three consultations held between one and four weeks apart, with measurements taken following the correct technique and using the mean of the last two of three readings taken in the consultation. Whenever possible, the diagnosis should be confirmed with out-of-office blood pressure measurements with either HBPM (Home blood pressure monitoring) or ABPM (ambulatory blood pressure monitoring) to assess white coat hypertension and masked hypertension.¹⁻⁴

CLASSIFICATION Classify the degree of blood pressure based on BP measurements during the consultation as: optimal (<120 and <80 mmHg), normal (120–129 and 80–84 mmHg), normal-high (130–139 and/or 85–89 mmHg), grade 1 hypertension (140–159 and/or 90–99 mmHg), grade 2 hypertension (160–179 and/or 100–109 mmHg), or grade 3 hypertension (≥ 180 and/or ≥ 110 mmHg), in accordance with the European Society of Hypertension.¹

Table 2 Cardiovascular risk assessment.**INITIAL APPROACH TO CARDIOVASCULAR RISK**

Assess cardiovascular risk in patients with hypertension and high normal blood pressure, as it influences therapeutic decisions. However, this should not delay the initiation of pharmacological treatment if required, especially in settings with limited resource availability.⁵

Cardiovascular risk assessment should be estimated based on cardiovascular risk factors, identification of hypertension-mediated organ damage, and associated clinical conditions such as established cardiovascular disease or chronic kidney disease.⁵

In patients who do not initially qualify as high or very high risk due to a history of established cardiovascular disease, moderate-to-severe chronic kidney disease, hypertension-mediated organ damage, diabetes mellitus, or familial hypercholesterolemia, the use of scoring systems that assess 10-year cardiovascular risk, such as SCORE 2 or the HEARTS scoring system, are recommended.⁶

Table 3 Initial routine tests and additional studies.**ROUTINE TESTS**

Clinical laboratory testing: it is recommended to include the following in routine blood tests: complete blood count, blood glucose, total cholesterol, HDL cholesterol, LDL cholesterol, triglycerides, serum creatinine, estimated glomerular filtration rate, serum urea, serum electrolytes, serum calcium, thyroid hormones, complete urinalysis, and albumin/creatinine ratio in a spot urine sample.^{3,7,8}

Electrocardiogram: A 12-lead electrocardiogram (ECG) is recommended for all patients with hypertension to detect left ventricular hypertrophy (LVH), arrhythmias, or ischemic heart disease.^{1,3}

Depending on the availability of resources, echocardiography is suggested as it provides information on ventricular and atrial dimensions and can evaluate left ventricular systolic and diastolic function, which is extremely useful in assessing hypertension-mediated organ damage.^{1,3,8}

ADDITIONAL TESTS

Additional specific, non-routine studies can be considered depending on clinical suspicion based on signs and symptoms, findings on basic studies, as well as the need to provide data on risk stratification that may modify behavior, according to availability.¹⁻⁴

hypertension during hospitalization. In addition, its final recommendations are applicable in all countries with participating scientific societies.

The importance of this position lies in having a single approach to HT in a common language that takes into account both the integral vision of internists in different clinical scenarios and in the context of different comorbidities as well as the resources available in Latin American countries.

The main limitation is the difficulty in widespread implementation of these recommendations. At present, there are already numerous clinical practice guidelines on HT. Although they have important similarities, some have cer-

tain differences in the cut-off values for diagnosis as well as semantic differences in classification that could lead to confusion among users.

The available evidence in this field of knowledge increases exponentially in very short periods of time. The authors believe that a comprehensive review of this document will be necessary in a maximum of two years.

In conclusion, this document presents the FIMI recommendations for the diagnosis and treatment of HT with the aim of guaranteeing effective and safe healthcare in the different areas of care for this disease.

Table 4 Treatment of hypertension.

THRESHOLDS FOR STARTING TREATMENT In patients between 18 and 79 years of age, it is recommended that pharmacological treatment, together with lifestyle changes, be started once a diagnosis has been made when the BP measured in the consultation room is higher than 140/90 mmHg.^{7,8} In patients with normal-high blood pressure (130–139/85–89 mmHg) who also have established cardiovascular disease, severe chronic kidney disease, or a very high cardiovascular risk scale score, pharmacological treatment is recommended.^{1,7} In patients 80 years of age or older, the decision to initiate treatment should be individualized and thresholds in the range of 140/150 mmHg SBP may be considered depending on clinical condition, frailty, and treatment tolerance.^{1–3} In cases where criteria for moderate or severe frailty are present, clinical judgment should be used to assess the need for antihypertensive treatment.³

THERAPEUTIC OBJECTIVES An optimal target, whenever possible and well tolerated, is to reduce blood pressure to the range of 120–129 and 70–79 mmHg. In people between 65 and 79 years of age, the aforementioned objective should be assessed and, if it is not possible, figures as close as reasonably possible to this optimal objective should be sought.⁷ During treatment, it is not recommended to seek a diastolic blood pressure lower than 70 mmHg.⁴ In patients older than 80 years without frailty, a target BP < 140/90 mmHg is recommended (DBP decrease should not be less than 70–65 mmHg), assessing tolerance to treatment and the presence of orthostatic hypotension during follow-up. In this age group, clinical judgment should be used if lower SBP ranges should be sought based on the patient's clinical status and cardiovascular risk, with assessment of treatment tolerance and potential adverse events.^{2,3,7}

TREATMENT OF HYPERTENSION Following a healthy lifestyle is recommended for all patients regardless of blood pressure and cardiovascular risk.^{1,3,7,8} Among the antihypertensive drugs that have demonstrated therapeutic efficacy and a reduction in cardiovascular events that are indicated as a basis for treatment, there are thiazide diuretics; renin angiotensin system blockers (RAAS), such as angiotensin-converting enzyme inhibitors (ACE inhibitors) or angiotensin II receptor blockers (ARB); and dihydropyridine calcium channel blockers.^{1,3,7,9} Beta-blockers can be used either in monotherapy or in combination when the clinical situation requires it or when there is another clinical indication in which their use would be beneficial.^{1,7} It is recommended to start combined therapy and in a single pill, if available, in most patients. As exceptions to combined therapy, patients with grade 1HT that is nearly borderline (less than 150/95 mmHg) with low cardiovascular risk (CVR), patients with high normal BP who require pharmacological treatment due to high CVR, or patients over 80 years of age, or frail patients who have opted to initiate pharmacological treatment could choose to start treatment in monotherapy.^{1,9} Drug combinations should preferably include an ACEI or ARB with a dihydropyridine calcium antagonist or a thiazide or thiazide-like diuretic.^{1–4,7,9} The pharmacological association of both compounds—ACEI and ARB—is not recommended.^{1–4,7,9} If BP is not adequately controlled with dual therapy, the patient should progress to triple therapy (ACEI or ARB + calcium antagonist + thiazide diuretic or thiazide-like diuretic).^{1–3,7,9} Beta-blockers can be combined when other clinical conditions require their use, such as coronary artery disease, heart failure, or heart rate control, among others.^{1,3,7–9} There is no contraindication for any of the aforementioned classes of first-line antihypertensives in individuals with diabetes, but ACEI or ARB are especially useful given the increased risk of proteinuria or microalbuminuria. Therefore, it is recommended that they be prioritized in the choice of combination therapy.^{1,3,7} In patients with chronic kidney disease, adequate antihypertensive treatment contributes to slowing the progression of renal failure, so the reduction of microalbuminuria should also be considered a therapeutic objective. Drugs that act on the renin angiotensin system (RAAS) are the first choice since they reduce microalbuminuria and the progression of renal damage.^{2,3,5}

TITRATION PHASE

[•]When possible and when clinical conditions permit, dose titration should be performed after two weeks, since this is the time it takes for some drugs to reach their maximum antihypertensive effect. Therefore, the patient should preferably be seen one month after starting treatment to assess treatment adherence and evaluate BP targets during the titration phase, with the exception of severe hypertension or clinical conditions that warrant shorter periods of time. Regarding final attainment of the BP target, there should generally be a delay of no more than three months, regardless of the number of times dose titration is required.¹

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Table 5 Resistant hypertension.**DEFINITION AND GENERAL CONCEPTS**

Patients with true resistant hypertension are individuals who do not achieve therapeutic objectives despite treatment consisting of lifestyle changes and the use of three first-line antihypertensive drugs at adequate doses, including a diuretic. It is corroborated by ambulatory BP measurements and after having ruled out the lack of treatment adherence.^{1,10}

It is recommended to measure BP outside the clinic, preferably through ambulatory blood pressure monitoring (ABPM) or self-measurement of blood pressure (SMBP) before classifying the HT as resistant in order to rule out white-coat hypertension. Secondary causes of hypertension should be ruled out in this group of patients.^{1,3,7,8,11}

Before defining a patient as resistant hypertensive, frequent situations of pseudo-resistance that influence poor BP control should be ruled out, such as lack of treatment adherence, measurement errors with inadequate or small cuffs in relation to arm circumference, inadequate combination of antihypertensive drugs, or drugs in sub-therapeutic doses.¹⁻⁴

TREATMENT APPROACH

Spironolactone is recommended as the fourth drug of choice in the vast majority of cases. For those who do not tolerate it, it can be replaced by eplerenone or amiloride.⁷

The combination with beta-blockers is suggested for patients with resistant HT in which the neurogenic mechanism plays an important role. Other antihypertensive drugs can also be considered depending on the clinical picture and lack of response.^{2,10}

Renal denervation has been positioned as a reasonable alternative that could be considered in reference centers only and after having analyzed the different options and in agreement with the patient. The choice of this method is not routine, but rather must be individualized and is contraindicated if the glomerular filtration rate is less than 40 ml/min.¹²

Due to the complexity that the overall management of patients with true resistant HT may require, referral to specialized hypertension centers or areas should be considered.^{1,7}

Table 6 Hypertension in patients hospitalized for non-cardiac and non-cerebrovascular causes.**HOSPITALIZED PATIENTS WITHOUT ACUTE TARGET ORGAN DAMAGE**

First, correct blood pressure measurement, a proper device, adequate cuff size, and proper arm and patient position must be evaluated.¹³

Factors that contribute to high blood pressure should be considered and treated. Multiple factors can elevate blood pressure in a hospitalized patient: pain, anxiety, nausea, hypo- or hypervolemia, and drug or alcohol withdrawal syndrome.

Hospitalization *per se* is a factor that produces biochemical alterations such as elevated cortisol and catecholamines and can lead to elevated blood pressure.^{13,14}

A review of outpatient medication and in-hospital medication is required since omission of the patient's baseline antihypertensive medication is a frequent cause of elevated blood pressure in the hospital. It should also be remembered that many medications frequently used in the hospital are associated with increased blood pressure, such as nonsteroidal anti-inflammatory drugs and corticosteroids.^{13,14}

It is not recommended to systematically intensify antihypertensive treatment or to do so in most asymptomatic patients who are hospitalized for noncardiac or non-cerebrovascular causes without acute target organ damage.¹³⁻¹⁵

At the time of discharge, it is advisable to ensure adequate follow-up of the hypertensive patient, scheduling a timely consultation with their internist or attending physician.^{13,14}

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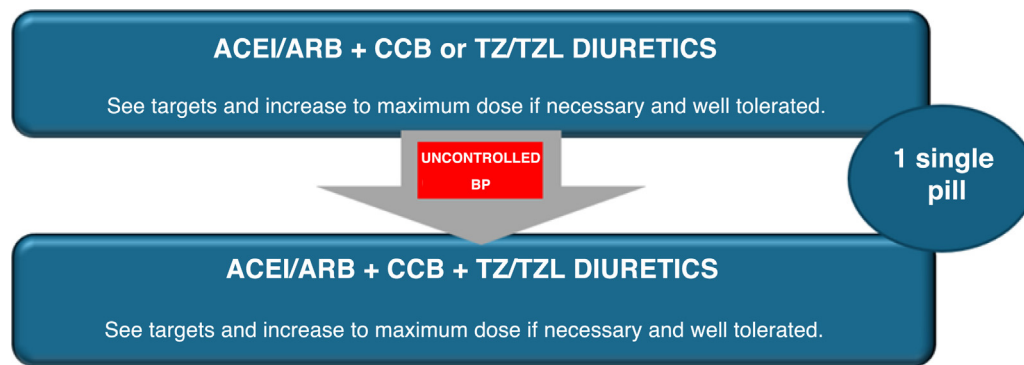


Figure 1 Recommended general pharmacological strategy.

Note: (a) Use of beta-blockers when required by the clinical situation. (b) Monotherapy can be initiated in patients with high normal BP but who require treatment due to very high cardiovascular risk, patients with grade 1HT close to the target (values below 150–90 mmHg) and with low cardiovascular risk and patients over 80 years of age or with frailty. (c) It is advisable to follow-up at one month during the titration phase and increase treatment if the targets are not met. (d) If targets are not met after triple therapy, an ABPM should preferably be performed (to rule out white-coat hypertension) and assess treatment adherence and secondary causes of HT. The fourth drug of choice is spironolactone and the management of these patients in specific services or centers should be considered. (see Table 5) ACEI: angiotensin-converting enzyme inhibitors; ARB: angiotensin II receptor blocker; CCB: calcium channel blockers; TZ: thiazide diuretics; TZL: thiazide-like diuretics.

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Declaration of competing interest

The authors declare that they do not have any conflicts of interest.

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